



Test Requisition Form

Exome sequencing

DRAFT · FOR CLINICAL & LEGAL REVIEW

Doc no: YLO/TRF/EXOME/00

Issue: 00 (draft)

Date: 08.10.2026 · Page 1 of 4

☐ Parental ☐ Postnatal

PATIENT DETAILS fill in block letters

☐ Mr. ☐ Ms. ☐ Mrs. ☐ Master FULL NAME _____

DATE OF BIRTH _____ AGE _____ SEX ☐ Male ☐ Female ☐ Transgender

ADDRESS _____

STATE _____ PINCODE _____ PHONE _____ EMAIL _____

CLINICIAN DETAILS fill in block letters

DOCTOR _____ HOSPITAL / LAB _____

PHONE _____ EMAIL _____ CLIENT CODE _____

DISEASE AFFECTATION STATUS ☐ Yes ☐ No AGE OF MANIFESTATION (YRS) _____ CONSANGUINITY ☐ Yes ☐ No CLINICAL HISTORY ATTACHED ☐ Yes ☐ No

ANY BLOOD TRANSFUSION ☐ Yes ☐ No DATE _____ ANY BONE MARROW TRANSPLANT ☐ Yes ☐ No DATE _____

I understand that this analysis is limited to molecular findings that relate to the symptoms in the clinical details I provided. Incidental findings that may not be actionable are not routinely reported, but can be provided on request after informed consent from the patient or guardian. New symptoms can change how a result is read, and a re-analysis may be needed at extra cost. I am authorised by the treating physician to order this test, and I confirm that the patient (or guardian of a minor) has been given full information about the test and its limits, in a language they understand.

CLINICIAN SIGNATURE

CLINICIAN STAMP

PLACE _____ DATE _____

TEST DETAILS

TEST CODE I _____ TEST NAME I _____

TEST CODE II _____ TEST NAME II _____

TEST CODE III _____ TEST NAME III _____

SPECIMEN DETAILS

Prenatal

- ☐ Amniotic fluid
☐ Chorionic villus sample
☐ Fetal cord blood
☐ Maternal blood (for MCC)
☐ Other

Product of conception

- ☐ Abortus tissue (with amniotic sac)
☐ Placenta (embryo side)
☐ Fetal cord blood
☐ Other

Postnatal

- ☐ Peripheral blood · individual · EDTA
☐ Peripheral blood · individual · heparin
☐ Peripheral blood · couple · EDTA
☐ Peripheral blood · couple · heparin
☐ Postnatal cord blood
☐ DNA storage
☐ Other

SANGER SEQUENCING FOR TARGETED VARIANT

GENE _____ VARIANT _____ TRANSCRIPT (NM ID) _____

PROBAND NAME _____ RELATIONSHIP WITH PROBAND _____ PREVIOUS REPORTS ATTACHED ☐ Yes ☐ No

SAMPLE COLLECTION

COLLECTED BY ☐ Hospital staff ☐ Collection centre staff ☐ Yello phlebotomist

DATE OF COLLECTION _____ TIME _____ NAME OF PERSON COLLECTING _____

PATIENT SIGNATURE _____

ACCOMPANYING SAMPLES if any

NAME	AGE	SEX	RELATIONSHIP WITH PATIENT

FOR LABORATORY USE ONLY

BOOKING ID _____

BARCODE

DOCKET NO _____ SAMPLE RECEIVED (DATE) _____ SAMPLE TYPE _____ VIALS _____

PEDIGREE AND FAMILY HISTORY

DESCRIBE AFFECTED RELATIVES, DRAW THE PEDIGREE, OR ATTACH

CLINICAL PHENOTYPES tick all that apply and add details

AUDITORY

- ☐ Sensorineural hearing loss · pre- / postlingual
- ☐ Enlarged vestibular aqueduct
- ☐ Microtia / anotia
- ☐ Ear tags / creases / preauricular sinus

CARDIOLOGY

- ☐ Congenital heart disease (septal / conotruncal / hypoplasia)
- ☐ Cardiomyopathy · dilated / hypertrophic / non-compaction / ARVC
- ☐ Arrhythmias · long QT / Brugada / short QT
- ☐ Echo findings
- ☐ ECG findings

DISORDERS OF SEX DEVELOPMENT

- ☐ Karyotype (specify)
- ☐ Ambiguous genitalia (specify)

DERMATOLOGY

- ☐ Albinism · ocular / oculocutaneous
- ☐ Ectodermal dysplasia
- ☐ Epidermolysis bullosa
- ☐ Ichthyosis
- ☐ Photosensitivity / keloids / lax skin
- ☐ Neurocutaneous markers

ENDOCRINE

- ☐ Diabetes · type 1 / type 2 / MODY / neonatal
- ☐ Early-onset hyperlipidaemia
- ☐ Hypothyroidism / Graves disease
- ☐ Hypo- / pseudohypoparathyroidism
- ☐ Hyperparathyroidism / pheochromocytoma / paraganglioma
- ☐ Adrenal insufficiency / CAH

GASTROINTESTINAL AND LIVER

- ☐ Neonatal cholestasis / liver failure
- ☐ Chronic liver disease
- ☐ Pancreatitis
- ☐ Malabsorption / diarrhoea
- ☐ Hepatosplenomegaly

HAEMATOLOGY / IMMUNOLOGY

- ☐ Anaemia (type)
- ☐ Bleeding disorder
- ☐ Recurrent infections
- ☐ Immunological work-up
- ☐ Bone marrow examination

MOVEMENT DISORDERS

- ☐ Ataxia · episodic / progressive
- ☐ Chorea / athetosis / dyskinesia
- ☐ Dystonia (site)

NEPHROLOGY

- ☐ CAKUT
- ☐ Haematuria / renal tubulopathy / nephrotic syndrome
- ☐ Cystic kidneys · ARPKD / ADPKD
- ☐ Renal biopsy findings

NEUROLOGICAL

- ☐ Developmental delay · global / motor / speech
- ☐ Intellectual disability · mild / moderate / severe
- ☐ Autism / stereotypies
- ☐ Neuroregression (age of onset)
- ☐ Seizures (type)
- ☐ EEG findings
- ☐ Recurrent headache / migraine
- ☐ Suspected inborn error of metabolism (attach report)

NEUROMUSCULAR AND AUTONOMIC

- ☐ Hypotonia · central / peripheral
- ☐ Weakness · proximal / distal
- ☐ Easy fatigability / myalgia / cramps
- ☐ Calf hypertrophy / scapular winging
- ☐ Contractures
- ☐ Joint laxity / dislocations
- ☐ Spasticity

BRAIN IMAGING

- ☐ Migration abnormality
- ☐ Calcifications
- ☐ Atrophy · cerebral / cerebellar
- ☐ Cerebellar hypoplasia

- ☐ Hypomyelination / demyelination
- ☐ Basal ganglia abnormality
- ☐ Holoprosencephaly / callosal agenesis
- ☐ Hydrocephalus
- ☐ Intraventricular haemorrhage

OPHTHALMOLOGY

- ☐ Cataract · congenital / uni- / bilateral
- ☐ Cloudy cornea / cherry-red spot
- ☐ Coloboma
- ☐ Glaucoma / buphthalmos
- ☐ Microphthalmia / anophthalmia
- ☐ Nystagmus / ptosis
- ☐ Optic atrophy / retinitis pigmentosa
- ☐ Retinoblastoma
- ☐ ERG / OCT findings
- ☐ Fundus examination

PERINATAL HISTORY

- ☐ Prematurity / birth asphyxia
- ☐ Teratogen exposure (specify)
- ☐ Maternal illness (specify)
- ☐ Oligo- / polyhydramnios
- ☐ Growth retardation
- ☐ Abnormal USG (specify)

SKULL AND HAIR

- ☐ Microcephaly · primary / secondary
- ☐ Macrocephaly
- ☐ Craniosynostosis
- ☐ Abnormal skull shape
- ☐ Encephalocele
- ☐ Hair · hypopigmented / sparse / absent

SKELETAL / LIMB

- ☐ Polydactyly
- ☐ Syndactyly / ectrodactyly / absent thumbs
- ☐ Limb hypoplasia / aplasia
- ☐ Micromelia / rhizomelia
- ☐ Metaphyseal / diaphyseal abnormality
- ☐ Osteopenia / fractures / osteopetrosis
- ☐ Spinal involvement

GENERAL INFORMATION ABOUT GENETIC TESTING

What is genetic testing?

Genetic conditions come from changes in a person's DNA. DNA carries the instructions for how the body grows and works. It is packed into 46 chromosomes, found in almost every cell. A gene is a stretch of DNA with the instructions for making a protein. Genetic testing looks for changes in chromosomes or genes. Its purpose here is to see whether the patient has a genetic change that explains their condition, or to estimate the chance of passing one on. In this form "patient" can also mean an unborn child. Your doctor can give more information about the specific test ordered.

What could I learn?

Please tell the laboratory if a gene change or chromosome rearrangement is already known in the family. A test can give one of three kinds of result:

- 1. Positive.** A gene or chromosome change was found that explains the condition, or that raises the chance of a condition in future.
- 2. Negative.** No disease-causing change was found by this test. This does not guarantee good health, and does not rule out a genetic cause the test could not detect.
- 3. Uncertain (variant of uncertain significance).** A change was found but it is not yet known whether it causes disease. It is not the same as a positive result. More tests on parents or relatives, and detailed medical records, may help. Medical knowledge changes, so the laboratory does not routinely re-analyse old results; you or your doctor can ask for a review.

Risks and limits

Genetic tests do not always give a clear answer. A change may exist and not be found, because of the limits of current knowledge and technology. If family relationships are not as stated, the result can be misread. Results are read together with clinical findings, family history and other tests, and only changes in genes linked to the patient's condition are reported. Rarely, results can be wrong, for example from a mislabelled sample, wrong clinical information, a rare technical error, or a recent bone marrow transplant or transfusion. The test cannot predict every long-term health risk. Some regions of the exome are not covered well enough to be read; coverage details are available on request. Large deletions or duplications, abnormal methylation, repeat expansions and similar changes are not routinely reported. A new sample may be needed if the first is not adequate. Turnaround time depends on the complexity of the test.

Prenatal samples

For prenatal testing, the fetal sample is checked for contamination with maternal cells. A maternal blood sample is strongly encouraged for this check.

Counselling

Genetic counselling before and after testing is recommended. The report does not diagnose a condition or recommend treatment; please review it with your doctor.



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INFORMED CONSENT

I have discussed this test with my doctor and understand what it involves and what it can and cannot tell us.

Required for testing

I agree to my sample and information being used by Yello Diagnostics to perform and report this test and to keep the records the law requires. I understand my results go to my ordering clinician, and are confidential.

Optional · research and future use

I may also agree to my **anonymised** data and leftover sample being used for research and for improving genetic knowledge for Indian populations. This is voluntary. Saying no does not affect my test or my care. If I agree, I understand that: my identity is removed before any sharing; I can withdraw at any time by contacting Yello Diagnostics through yello.health, and work already done will not be undone; and I will not be paid for, or hold ownership in, discoveries made. Fetal sex determination is not permitted in India and will not be reported.

RESEARCH AND FUTURE USE OF MY ANONYMISED DATA AND LEFTOVER SAMPLE ☐ I agree ☐ I do not agree

TEST SELECTION

- | | | |
|--|--|---|
| ☐ Clinical exome sequencing | ☐ Clinical exome · carrier (couple) | ☐ Whole exome + mitochondrial |
| ☐ Clinical exome · trio | ☐ Clinical exome · mitochondrial | ☐ Whole exome · trio |
| ☐ Clinical exome · carrier (single) | ☐ Targeted exome sequencing | ☐ Whole exome · carrier (single) |
| | ☐ Whole exome sequencing | ☐ Whole exome · carrier (couple) |

TEST CODES _____

DECLARATION

I have understood the above, or the staff have explained it to me or my child. **Select one:**

- ☐ For myself / on behalf of the person named, as I am legally authorised: I consent to this test.
- ☐ For a child aged 7-18: I am the parent or legal guardian and consent; the child agrees to take part.
- ☐ For a child under 7: I am the parent or legal guardian and consent.

PATIENT'S NAME _____

DATE _____

PATIENT / GUARDIAN SIGNATURE